



HEALTH CARE DELIVERY SOLUTIONS (HCDS) MHS GENESIS INDUSTRY DAY

DOD Healthcare Management System Modernization (DHMSM) Program Management Office (PMO)





VISITOR /SW
ENTRANCE I

DEFENSE HEALTH
HEADQUARTERS

7700 ARLINGTON BLVD



Meet Our Hosts



Dr. Christina Sheets
DHMSM Program Manager (PM)

Dr. Sheets is the DHMSM PM, overseeing the cost, schedule, and performance of MHS GENESIS. A retired Air Force Colonel, Dr. Sheets previously held various roles within DHMSM and served in dental leadership positions, bringing extensive experience to her current role.



Sonya Edom
Procuring Contracting Officer

Ms. Edom serves as a Contracting Officer supporting PEO DHMS and its Program Offices.

Email: Sonya.m.edom.civ@health.mil



Ground Rules



Listening Mode Only

We'll explain how to engage and provide feedback later in the briefing.



Submitting Input and/or Questions

Refer to Next Steps section



Presentation Materials Available

These slides are available on SAM.gov for your reference.

This session is being recorded and made available after today.



Purpose of Today's Session

Ensure high quality on the first submission – helping us move faster and respond more efficiently. Your input will support a more streamlined acquisition process from the outset.



Purpose

- Promote early industry understanding of mission, priorities, and constraints
- Enable meaningful vendor collaboration and informed proposal development
- Solicit feedback to strengthen acquisition outcomes and reduce delivery risk
- Foster a competitive environment that delivers value, innovation, and accountability
- Gather market insights to inform refinements prior to RFP release
- Clarify planned contracting approach and milestones
- Support industry readiness to respond with compliant, high-quality proposals



Agenda

➤ MHS GENESIS History & Background

Acquisition Strategy

Industry Input

Key Takeaways

Next Steps



MHS GENESIS History & Background

Overview

In 2015, the Government awarded a ten-year contract to implement MHS GENESIS. DHMSM designed, configured, and tested MHS GENESIS prior to deployment.

Moving towards Capability Support Phase

As of October 2025

Active projects in flight	41
DoW Military Medical Treatment Facility (MTF) Commands	138
Provisioned MHS GENESIS users	194K
Beneficiaries Served	9.6M
Total Locations	4386

Features

- Proven commercial EHR facilitates the **continuum of care** across garrison, operational, and enroute care
- Provides **world class healthcare delivery solutions** for Service Members, their families, and our Allies across the globe

Benefits

- Enables **critical information sharing across health care providers** such as the DOW, VA, and private sector healthcare
- Utilizes **industry-standard** longitudinal health care tracking to assist healthcare providers in making **high-quality, responsive, and personalized healthcare decisions**
- Allows greater access to personal healthcare information for patients and support **increased engagement in health-related activities**



MHS GENESIS History & Background

Priorities



DOW Priorities

- Revive the warrior ethos and restore trust in our military
- Rebuild our military by matching threats to capabilities
- Reestablish deterrence by defending our homeland—on the ground and in the sky

MHS Priorities

- Support the warfighter, ensures fitness and access to care
- Sustain clinical skills by increasing volume and complexity of care in our hospitals
- Strengthen our force generation platform with advanced training in our hospitals

DHA Priorities

- Assure the force is medically ready
- In partnership with the Joint Staff and Military Departments, assure our ability to generate and sustain the medical force
 - Increase operational clinical readiness (ready medical force)
 - Reattract beneficiaries to MTFs
- As a combat support agency, optimize support of the Joint Staff, Combatant Commands, and Military Departments

PEO Priorities

- Enhance patient care through continued evolution of MHS GENESIS and delivery of innovative health IT solutions
- Advance healthcare technology by implementing new capabilities and software releases that improve system performance and interoperability
- Sustain readiness and reliability through proactive technology refreshes and infrastructure optimization

DHMSM Priorities

- Field & optimize products that address and anticipate customer needs
- Accelerate productivity and effectiveness using the product platform model (people, process, tools, data)
- Shape and scale the innovation to enable the future now
- Partner with Industry, Federal Partners, stakeholders, and users to deliver value
- Invest in our people



MHS GENESIS History & Background

Operations



DOW/NSA

MTF Commands: **184**
Locations: **3,628**
Users: **176,391**



NOAA

- Locations: **7**
- Users: **17**



USCG

- Locations: **118**
- Users: **954**



USNS Mercy, San Diego, CA - MHS GENESIS Segment 2

- Locations: **1**
- Users: **47**



US MEPCOM

- Locations: **70**
- Users: **2,786**



RESERVES

- Locations: **562**
- Users: **5113**

- Messaging:
 - **23M** Daily
 - **65** external interfaces
 - **976** endpoints

- Provisioned Users: **194K**

- Beneficiaries: **9.6M**

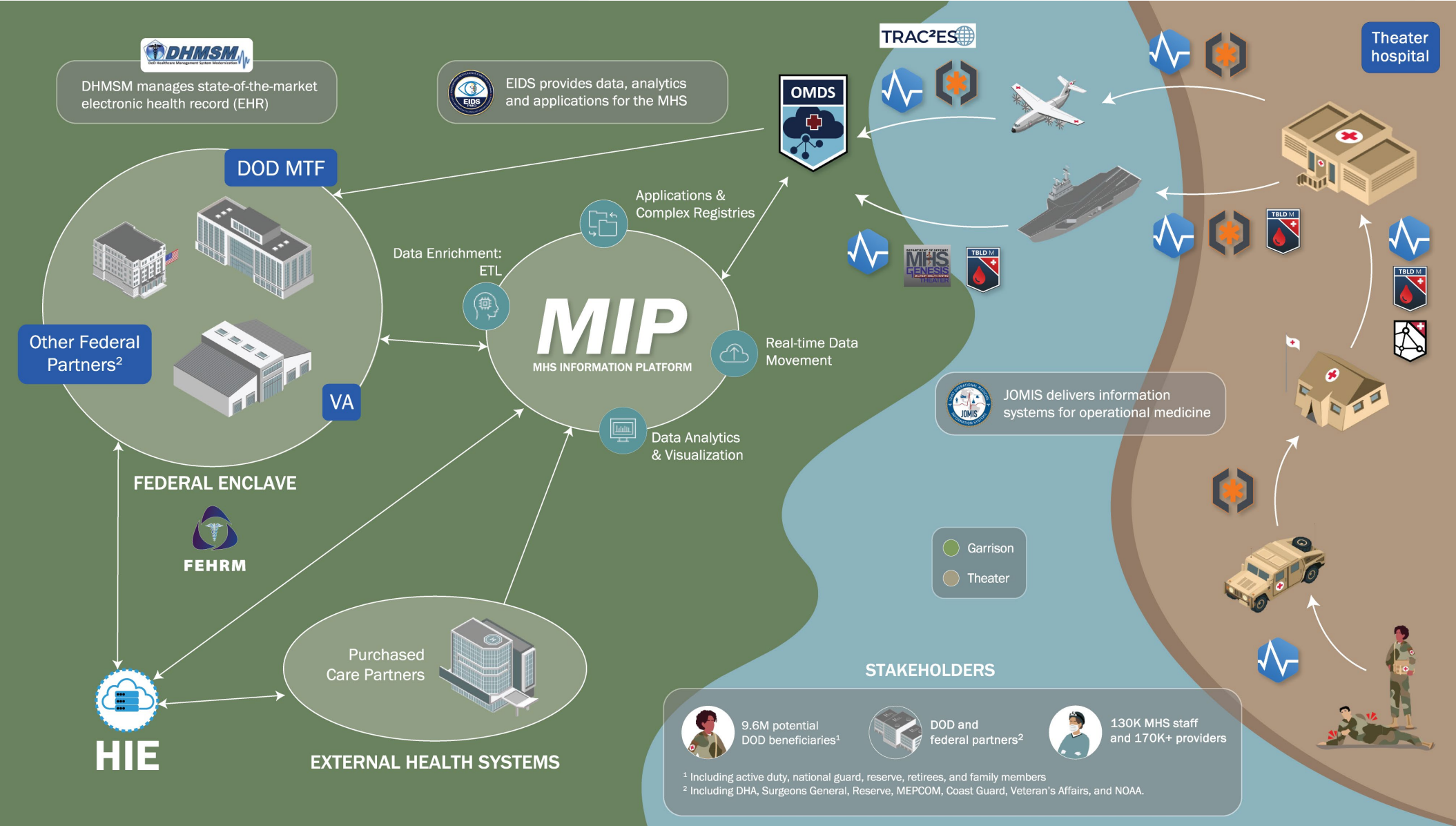
Future Deployment Sites

*USNS Comfort, Norfolk, VA - MHS GENESIS Segment 2 Deployment to Theater •
USCG Afloat System • Armed Forces Retirement Home*



MHS GENESIS History & Background

Across the Federal Space



9.6M potential DOD beneficiaries¹



DOD and federal partners²



130K MHS staff and 170K+ providers

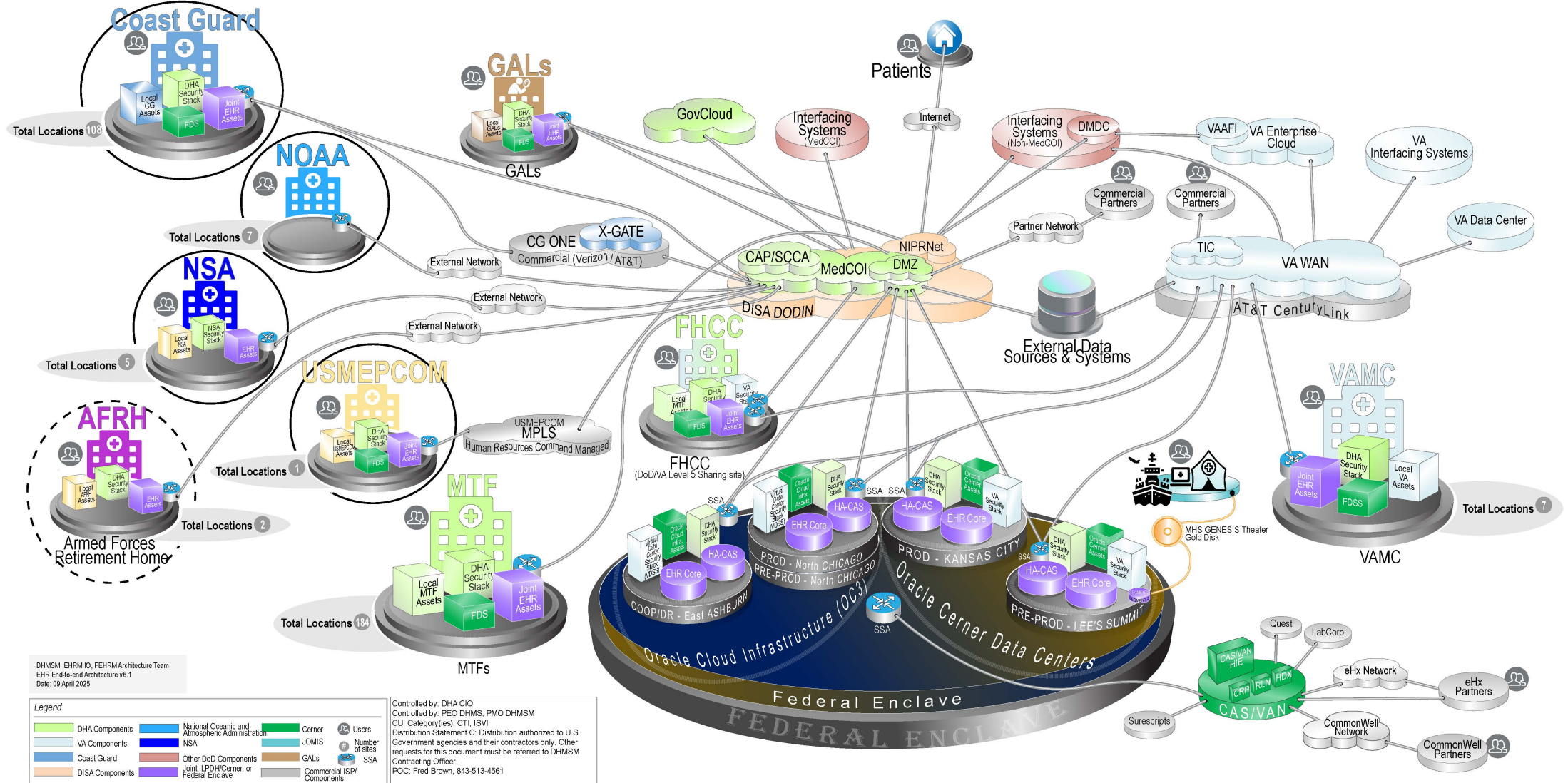
¹ Including active duty, national guard, reserve, retirees, and family members

² Including DHA, Surgeons General, Reserve, MEPCOM, Coast Guard, Veteran's Affairs, and NOAA.



MHS GENESIS History & Background

Medical Community of Interest (Med-COI) Network



The Med-COI network provides a secure connection framework from MHS GENESIS to DOW facilities and external interfaces 10



MHS GENESIS History & Background

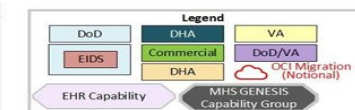
Capability Mapping

Federal EHR Capability to MHS GENESIS mapping underscores the complexity and depth of the system of systems

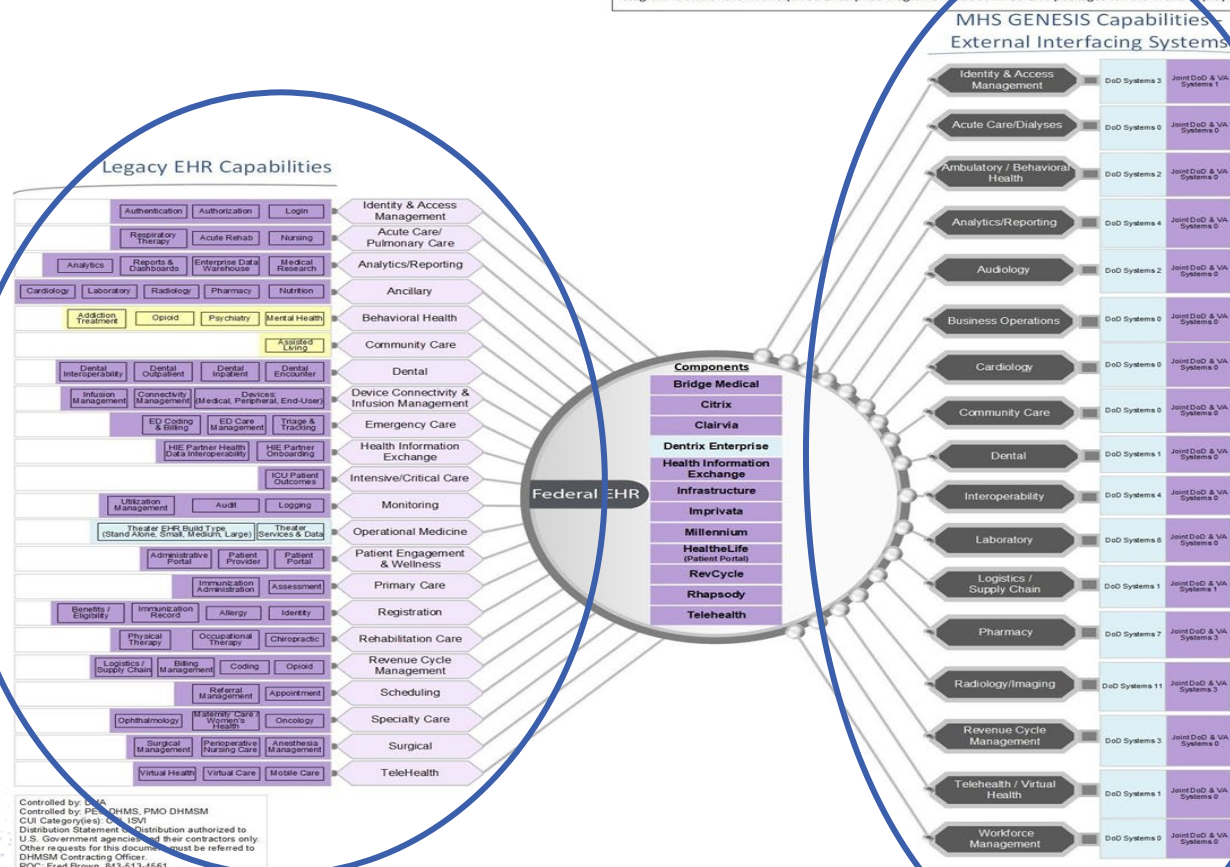
DOCUMENT CONFIGURATION		
Project: Federal EHR Enterprise Diagram – Fit-For-Purpose Presentation Diagram		
Diagram: Federal EHR Capability to MHS GENESIS Interface Mapping		
Version: 7.4	Date Initiated: 01 Dec 2022	DATE REVISED: 3 Nov 2025
Created By: Michael Johnson (mjohnson@ascr.federal.com)		
Government Lead: Mr. Fred Brown (fredrick.n.brown1.civ@us.navy.mil)		
Created With: Microsoft Visio Professional		
Technical POC(s): DHMSM – SE Architecture Team & LPDH		

DIAGRAM PURPOSE

The original purpose of this diagram was to map the Legacy EHR capabilities and Solution Categories (on the left in pink) to the current FEHR MHS GENESIS Capability groupings (on the right) and approved by the DHA Stakeholders. However, by popular demand, the original scope has been enhanced and expanded overtime to accommodate the external interfacing systems i.e., DOD, Department of Veterans Affairs (VA) and Commercial systems/entities. These interfaces are grouped by MHS GENESIS capabilities and do not reflect the Application Programmable Interfaces (APIs), Gateways, Network Components/services or the Medical Devices etc., For more information on technical specifications of these external systems interfaces, please review their respective Interface Control Document (ICD) and Federal Enterprise Electronic Health Record (FEHR) Enterprise System View (SV-1). All the revisions and modification are made in close collaboration with DHMSM Functional SMEs and LPDH System Engineers and updated quarterly and in conjunction with LPDH's System Subsystem Design Description (SSDD) delivery schedule. Also, this diagram is one of the five required Enterprise diagrams included in Go-Live packages for the Wave deployments.



Government requirements:
MHS GENESIS satisfies some or all of the 60 capabilities



Major solution components:

- Oracle Health Millennium – EHR
- Cerner Patient Accounting – Revenue Cycle
- Dentrix – Dental
- Rhapsody – Integration Engine
- Phillips eCare – Virtual Health Delivery
- Amwell Converge – Virtual Health
- Health Data Intelligence – Population health management and analytics
- Solventum Health Information Systems – AI and Integrated solution interoperability between DOW and VA
- Artera – On-demand pharmacy refills

Configuration Facts:

- 975 number of workflows
- 211 number of user roles
- 3000+ change requests annually



Agenda

MHS GENESIS History & Background

➤ **Acquisition Strategy**

Industry Input

Key Takeaways

Next Steps



Acquisition Strategy

DHMSM Guiding Principles



Standardization of clinical and business processes across the Services and the Military Health System (MHS)



Design a patient-centric system focusing on quality, safety and patient outcomes that meet readiness objectives



Flexible, open, and **interoperable system of enterprise solutions that support the full patient continuum of care**



Maximizing value through business process reengineering, adoption, and implementation over technology



Leverage transformative technology advancements to provide continuous improvement and state-of-the-art patient care



Decision-making and design will be driven by frontline care delivery professionals



Decisions shall be based on doing what is best for the MHS as a whole – not a single individual area



Drive towards rapid decision making to keep the program on time and on budget



Provide timely and complete communication, training and tools to **ensure optimal Stakeholder Experience**



Build collaborative partnerships outside the MHS to advance national interoperability



Enable full patient engagement in their health **by prioritizing their User Experience**



Enhance patient safety through solutions that reduce harm, improve care, reduce waste, and improve efficiency



Acquisition Strategy

Initial Strategy

Initiative	Requirements	Key Considerations
System Integration	• Operate and maintain system, including configuration, patches, system security, & cybersecurity compliance • Provide the supported solutions and maintain licenses • Manage sub-contractor performance • Focus on optimization and efficiency	Due to complexity of MHS GENESIS and the on-going hosting transition from on-premises to cloud, the risk associated with transitioning contractors was significantly high and would impact the success of cloud transition
End User Experience	• Training, Change Management and user adoption • Tier 1/ Tier 2 help desk support • Routine fixes and user-centric EHR configurations.	Competition introduces low risk and allows for tailored vendor selection
New / Enhanced Capabilities	• Deliver new capabilities or solutions to meet requirements • Implement new capabilities to enterprise to include other Federal Partners (not Deployment)	Competition introduces low risk and allows for tailored vendor selection
Deployment	• On-board new Federal Partners	Competition introduces low risk and allows for tailored vendor selection



Acquisition Strategy Update

The Government is conducting additional market research and industry engagement to inform a revised approach that reflects emerging requirements and priorities.

- **Strategic Alignment Opportunity**
 - The Department is exploring a joint DOW/VA contract for the EHR system, signaling a move towards tighter Federal integration.
- **Value Optimization Focus**
 - Drive value optimization by aligning investments and capabilities to measurable mission outcomes, eliminating low-yield activities, and accelerating delivery of impactful solutions at reduced total cost of ownership.
- **Prioritize Innovation**
 - Shift value optimization approach to embrace calculated risk, enabling faster innovation and unlocking higher-return opportunities.



Acquisition Strategy

Update

The Government is conducting additional market research and industry engagement to inform a revised approach that reflects emerging requirements and priorities.

- **Leveraged IDIQ for MHS GENESIS Sustainment**
 - Notwithstanding the publication of a notice of intent to sole source, the Government awarded Stage 11 to continue “mostly as-is” sustainment support for 12 months (29 July 2025 – 28 July 2026).
 - Awarded an additional Product Improvement Engineering Task Order to support emerging new requirements for completion by 28 July 2026.
- **Planning for a new, competitive contract**
 - Develop and align the acquisition strategy, launch RFP development, and coordinate organizational readiness in parallel.



Acquisition Strategy

Key Takeaways from Existing Market Research

- Accelerating healthcare innovation reshapes deployment and value.
- Vendor relationships with shared outcomes and accountability with measurable Key Performance Indicators (KPIs).
- Emphasis on speed-to-delivery, interoperability, and incremental adoption of modular capability.
- A defined Target State Architecture (TSA) to inform decisions.
- Balancing “optimize today” versus “transform for tomorrow” mindset.
- Collaborative partnerships with shared accountability and transparency.
- Transitioning requires a rigorous knowledge transfer, deep and broad expertise, and a well-managed transition period.
- Shift towards performance-based compensation models that share risk and reward.



Acquisition Strategy

Future State & Priorities



Performance



Continuous Capability Delivery

Competitive landscape (cost, technical, and return on investment)



Cost Efficiency



Agenda

MHS GENESIS History & Background

Acquisition Strategy

➤ **Industry Input**

Key Takeaways

Next Steps



Industry Input

Transition Strategy

The Government is seeking industry feedback on the contracting approach on how to best optimize a transition to mitigate impact to operations and cost:



Are there other major solution components the Government should consider? If so, explain how they meet the Strategic Alignment Opportunity while maintaining performance, cost, and competition goals.



Given the Government's constraints, how would you best manage the existing major solution components?



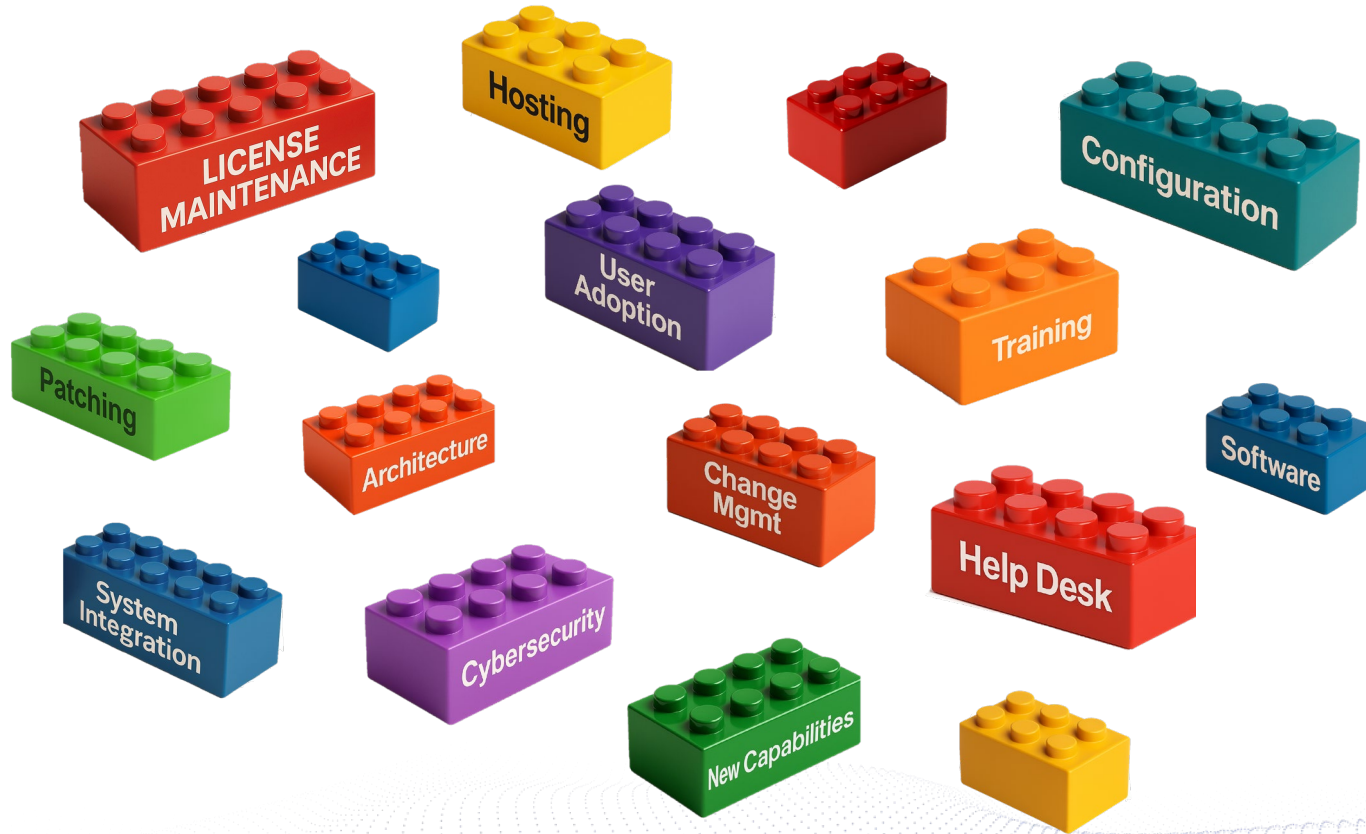
In order to create a more competitive environment, should there be a transition from the existing prime contractor structure into multiple contracts?



Industry Input

Organizational Construct

Build organizational structures that allows for informed decision-making and maximum efficiency.



Group capabilities
together for
performance, cost,
efficiency, and to
optimize competition



Industry Input

Role and Value of a System Integrator(s)

1

Sustain competition throughout the portfolio

2

Organize to maximize value and performance

- What is the ideal relationship among contractors/vendors
- Vendor integration models

3

Mitigate Risk for limitations of the Government

4

Is there a value proposition for a System Integrator(s)



Industry Input

Near Term Outcomes



Healthcare Outcomes



Technology as a Value Instrument



Establish KPIs





Industry Input

Compensation Plan



In a resource constrained environment, the Government seeks an approach to compensation that drives value, performance and outcomes



Compensation based on results, not labor



Model should account for:

- Objectively measured outcomes
- Priority alignment
- Maintaining competition



Is there a process that objectively measures the value of capabilities and leverages modular capability to drive outcomes and performance?



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➤ **Key Takeaways**

Next Steps



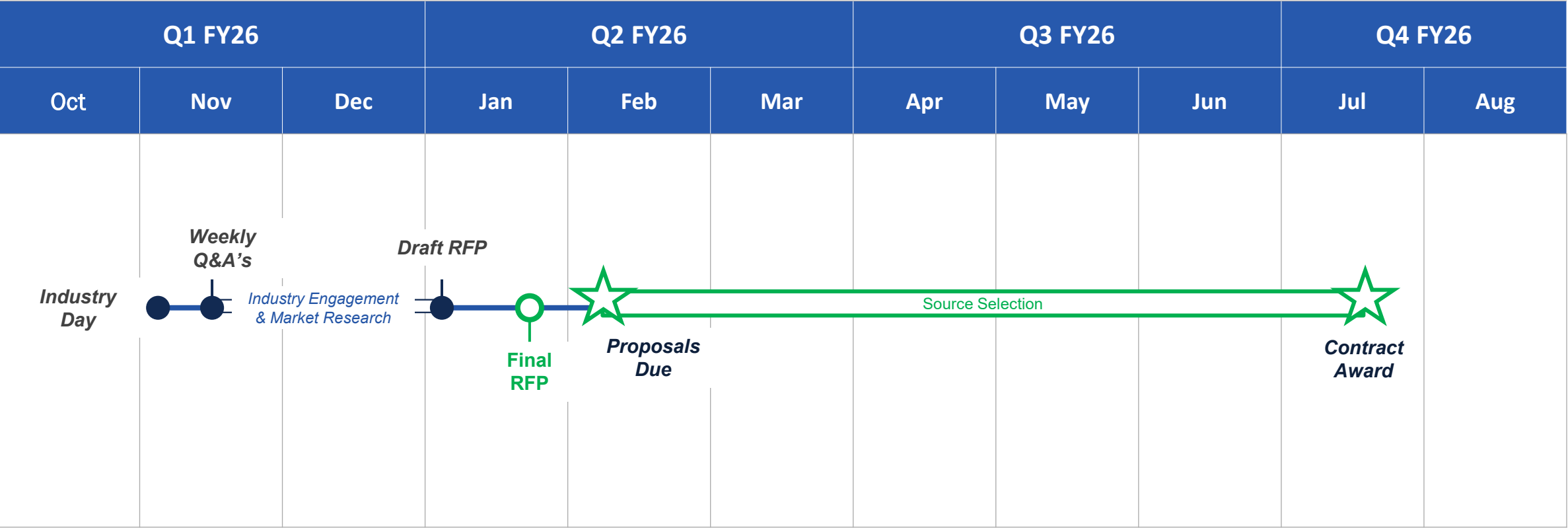
Key Takeaways

- Mutually Beneficial Partnership
- Industry Engagement Drives Refinement
 - Feedback from today informs the Government's approach, acquisition strategy, and RFP
- Drive Value Optimization by Aligning Investments to Mission Priorities
 - Eliminate low-yield activities and accelerate delivery of impactful solutions at reduced total cost
- Focus on Federal Partner Synergy
 - Greater interoperability, strategy, and cost efficiencies
- Optimize Competition
 - Structure work to balance cost, performance, and innovation



Key Takeaways

Estimated Timeline



As of September 30, 2025



Key Takeaways

Guidance from Contracts

- **Official Information Only**
 - If you're not hearing it directly from the Contracting Officer or SAM.gov, consider it speculation.
- **Transparency Commitment**
 - We are committed to providing clear and timely information.
- **No "Inside Track"**
 - There are no shortcuts to accessing legitimate contract details. All information will be disseminated fairly and openly.
- **"Ask, Don't Assume"**
 - If anything is unclear, please use the official Q&A channels provided. Don't rely on unverified sources.
- **We Are Your Source**
 - We are working diligently to finalize details and will be your definitive source of information for this contract.
- **Be Patient**
 - We understand the eagerness for information and appreciate your patience as we work through the process.



Agenda

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➤ **Next Steps**



Next Steps

Provide Feedback

- **Industry Input**
 - Please provide feedback to dha-dhms-cd@health.mil no later than **Monday, November 17th** (sooner is preferred)
 - Multiple e-mails are okay
 - If you provide feedback in multiple emails, please indicate pertinent information on the Subject line
- **Questions**
 - Submit questions in writing via Bidscale (see next slide)
 - Questions must be within scope of the material covered at Industry Day
 - Copies of all pertinent questions and answered will be posted on Sam.gov weekly
 - The questions and answers will inform the development of the RFP.
- **Today's presentation and video recording link will be posted to SAM.gov**



Instructions to Submit Questions and/or Feedback

- **Request User Account (only one request per vendor)**
 - Send an email to customersuccess@bidscale.com. The email must include the following information:
 - Subject Line: Submission Portal Account Creation
 - First Name
 - Last Name
 - Email Address
 - Company Name
 - HCDS EHR Follow-on (MHS GENESIS)
- **Confirm Email & Login**
 - Once the account is created, vendors will receive a confirmation email (within one business day) with login instructions.
- **Submit Questions and/or Feedback**
 - File names must be lowercase alphanumeric only and include the date of submission formatted DDMMYYYY.
 - Submit only when ready; files may be immediately distributed to the government.
 - Supported web browsers include Chrome, Edge, and Firefox.

Bidscale is only authorized in a limited capacity to issue access to the submission portal and provide basic technical support with its utilization. If a confirmation email was not received or technical issues occur with the submission portal, contact Bidscale for assistance at customersuccess@bidscale.com.



THANK YOU